DIAGNOSTIC REPORT

MC-5725

agilus diagnostics

PATIENT NAME : SHREETIMAMA MOHANTA **REF. DOCTOR :** SELF

CODE/NAME & ADDRESS : C000136336 HEALTH POINT KALINGA NAGAR, K-3,KHORDA BHUBANESWAR 751019 9937146448	ACCESSION NO : **0310YA009664** PATIENT ID : SHREF260192310A CLIENT PATIENT ID: ABHA NO :	AGE/SEX : 33 Years Female DRAWN : 26/01/2025 00:00:00 RECEIVED : 26/01/2025 14:09:17 REPORTED : 26/01/2025 17:49:02

Test Report Status **Preliminary**	**Results**	**Biological Reference Interval**	**Units**
SEROLOGY			
WIDAL TEST, SERUM			
S TYPHI O ANTIGEN TITRE METHOD : SLIDE AGGLUTINATION	**1:80**	< 1:80	TITER
S TYPHI H ANTIGEN TITRE METHOD : SLIDE AGGLUTINATION	< 1:80	< 1:80	TITER
S PARATYPHI AH ANTIGEN METHOD : SLIDE AGGLUTINATION	< 1:80	< 1:80	TITER
S PARATYPHI BH ANTIGEN METHOD : SLIDE AGGLUTINATION	< 1:80	< 1:80	TITER
MALARIA ANTIGEN DETECTION, WHOLE BLOOD			
PLASMODIUM FALCIPARUM ANTIGEN METHOD : IMMUNOCHROMATOGRAPHY	NOT DETECTED	NOT DETECTED	
PLASMODIUM VIVAX ANTIGEN METHOD : IMMUNOCHROMATOGRAPHY	NOT DETECTED	NOT DETECTED	

Interpretation(s)
WIDAL TEST, SERUM-The Widal agglutination test is used for diagnosing Enteric Fever. The term enteric fever includes typhoid fever caused by Salmonella typhi, Salmonella paratyphi A, B and C. Though enteric fever is endemic in all parts of India, S.paratyphi C infections are uncommon and are not included in Widal testing.

Test Utility:
The Widal test measures the antibodies against the H (flagellar) & O (somatic) antigens of typhoid and paratyphoid(A & B) bacilli ,in the patients sera. The test is performed in serially increasing dilutions.
- Diagnostic titre of Widal test varies highly between different geographical locations. It depends upon the baseline titre prevalent amongst the healthy individuals in that geographical area, which in turn is influenced by endemicity of typhoid in that region.
- The titre of the Widal test will depend on the stage of the disease. Antibodies usually appear by the beginning of second week of infection. Hence blood taken earlier may give a negative result. The titre increases steadily till the 3rd or 4th week after which it declines gradually.
- Cases treated early with antibiotics may show a poor antibody response.
- A single Widal test is of little clinical relevance due to the number of cross reacting infections,including malaria,tuberculosis, pneumonia, amoebiasis, rickettsial disease, Rheumatoid arthritis, hepatitis B. A fourfold increase in the titer in paired sera in the course of the infection would be consistent with a typhoid infection.
- Persons who have suffered from enteric fever in the past may show agglutinins in moderate titre ,even when suffering from other unrelated illness. Such anamnestic appearance may be differentitated by repeat testing after 7-10 days. Anamnestic response will show only a transient rise, while in enteric fever the rise will be sustained.
- TAB vaccinated patients may show a moderate rise in the titres against all three 'H', 'AH' & 'BH' antigens.

MALARIA ANTIGEN DETECTION, WHOLE BLOOD-Four species of the plasmodium parasites are responsible for human malaria infections P. falciparum, P. vivax, P. ovale and P. malariae. P. falciparum and P. vivax are the most prevalent. Early detection and differentiation of malaria is of paramount importance due to incidence of cerebral malaria and drug resistance associated with P. falciparum malaria causing most of the morbidity and mortality worldwide .As treatment depends on the species, differential diagnosis of P. falciparum and P. vivax is extremely important for better patient care management and faster recovery.

Test Utility:
The current test is a qualitative test for detection of the P. falciparum specific histidine rich protein-2 (Pf. HRP-2) and P. vivax specific lactate dehydrogenase (pLDH) in whole blood samples. The assay is able to detect and distinguish P. vivax and P. falciparum infections and also identify mixed infections.

Notes & Limitations:

Dr.Sabana Parwin MBBS, M.D
(Pathology)
Lab Head

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ULR No.775000010939954-0310

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- In case of a suspected P. falciparum case, a positive band for Pf. HRP-2 is highly predictive of infection, but a negative test result may not rule out the same. This may be seen in patients with i) very low parasitic index (<5 parasites/HPF), as the test sensitivity and test band intensity correlated directly with the parasite density, and i) Carriers, as Pf. HRP-2 is not secreted in the gametogony (sexual) stage. Rarely, a false negative result may be seen in individuals with a gene deletion for the production of HRP-2.
- Pf. HRP-2 persists after successful therapy and may be detectable even after the clinical symptoms of malaria have disappeared and the parasites have apparently been cleared from the host. Hence it should not be used for monitoring early therapeutic responses.
- In case of a stand alone P. vivax infection, the band for pLDH may be used to monitor patient progress during therapy and predict recrudescence and possible drug-resistant infections. In case the pLDH band remains positive with the same intensity after 5-10 days, the possibility of a resistant strain has to be considered. A negative pLDH result does not rule out P. vivax infection, as may be seen in patients with a very low parasitic index (<5 parasites/HPF).
Since the test detects P. falciparum specific HRP-2 and P. vivax specific LDH, a negative test result does not rule out infection with P. ovale and P. malariae.
- Constant exposure to the malarial parasites, as seen in areas of high endemicity, may result in positive results with doubtful clinical significance. Hence, the results must always be correlated with clinical history and relevant epidemiological and therapeutic context. Also note that blood film examination remains the standard method for diagnosing malaria, since it detects all Plasmodium spp. and allows visualization of parasite growth stages, which is essential for making therapeutic decisions.

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MICRO BIOLOGY

CULTURE, URINE WITH SUSCEPTIBILITY	RESULT PENDING

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CLINICAL PATH - URINALYSIS

URINALYSIS

PHYSICAL EXAMINATION, URINE

COLOR	PALE YELLOW		
APPEARANCE	CLEAR		

CHEMICAL EXAMINATION, URINE

Test	Results	Biological Reference Interval	Units
PH	6.0	4.7 - 7.5	
SPECIFIC GRAVITY	1.005	1.003 - 1.035	
PROTEIN	NOT DETECTED	NOT DETECTED	
GLUCOSE	NOT DETECTED	NOT DETECTED	
KETONES	NEGATIVE	NOT DETECTED	
BLOOD	NEGATIVE	NEGATIVE	
BILIRUBIN	NEGATIVE	NOT DETECTED	
UROBILINOGEN	NORMAL	NORMAL	
NITRITE	NEGATIVE	NOT DETECTED	
LEUKOCYTE ESTERASE	NOT DETECTED	NOT DETECTED	

MICROSCOPIC EXAMINATION, URINE

Test	Results	Biological Reference Interval	Units
RED BLOOD CELLS	NOT DETECTED	NOT DETECTED	/HPF
PUS CELL (WBCS)	1-2	0-5	/HPF
EPITHELIAL CELLS	2-3	0-5	/HPF
CASTS	NOT DETECTED		
CRYSTALS	NOT DETECTED		
BACTERIA	NOT DETECTED	NOT DETECTED	
YEAST	NOT DETECTED	NOT DETECTED	

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****End Of Report****

Please visit www.agilusdiagnostics.com for related Test Information for this accession

CONDITIONS OF LABORATORY TESTING & REPORTING

1. It is presumed that the test sample belongs to the patient named or identified in the test requisition form.
2. All tests are performed and reported as per the turnaround time stated in the AGILUS Directory of Services.
3. Result delays could occur due to unforeseen circumstances such as non-availability of kits / equipment breakdown / natural calamities / technical downtime or any other unforeseen event.
4. A requested test might not be performed if:
   i. Specimen received is insufficient or inappropriate
   ii. Specimen quality is unsatisfactory
   iii. Incorrect specimen type
   iv. Discrepancy between identification on specimen container label and test requisition form
5. AGILUS Diagnostics confirms that all tests have been performed or assayed with highest quality standards, clinical safety & technical integrity.
6. Laboratory results should not be interpreted in isolation; it must be correlated with clinical information and be interpreted by registered medical practitioners only to determine final diagnosis.
7. Test results may vary based on time of collection, physiological condition of the patient, current medication or nutritional and dietary changes. Please consult your doctor or call us for any clarification.
8. Test results cannot be used for Medico legal purposes.
9. In case of queries please call customer care (91115 91115) within 48 hours of the report.

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